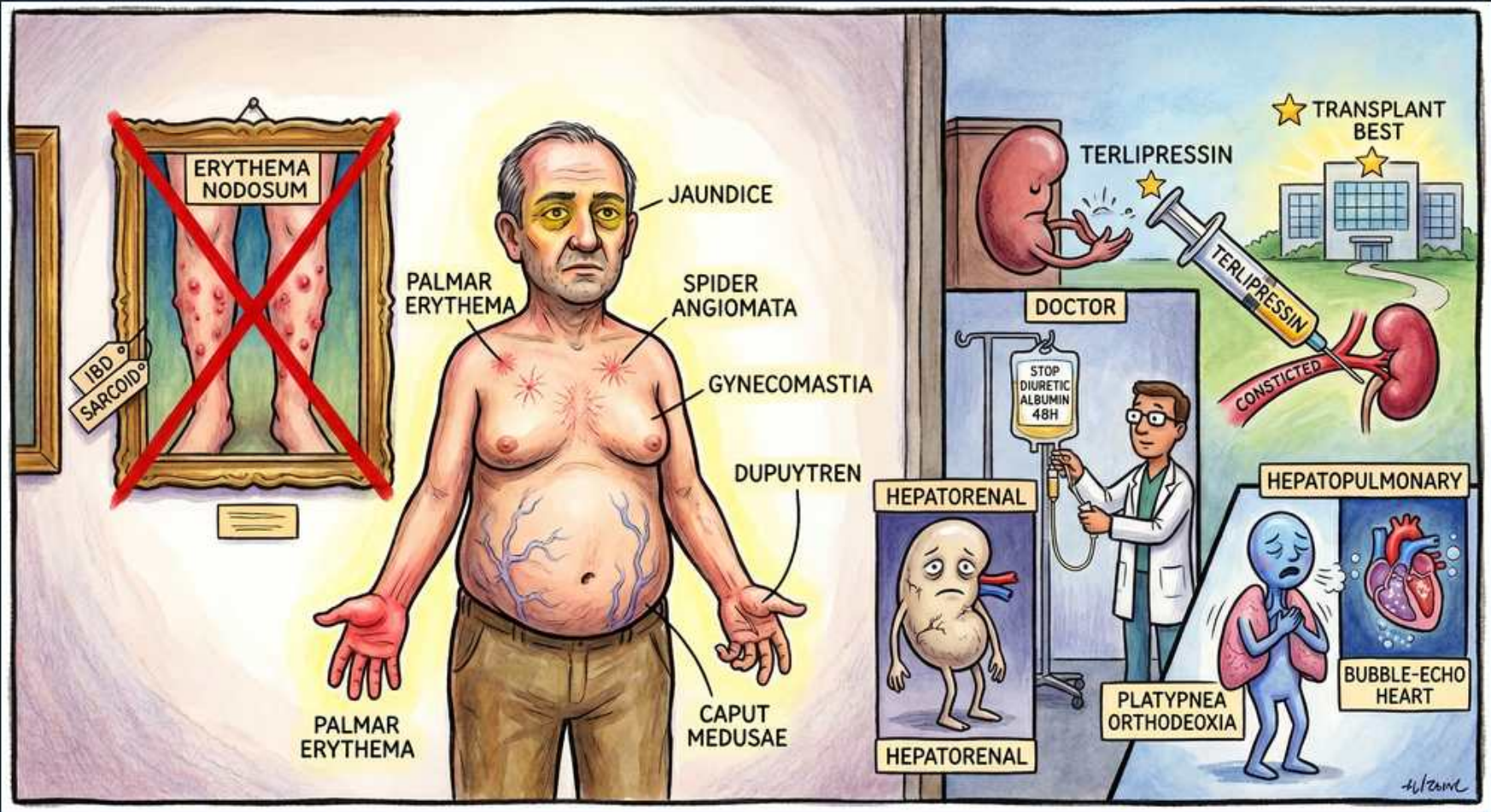


Cirrhosis — Stigmata, HRS & Hepatopulmonary



1. Erythema nodosum (X)

WHAT YOU SEE

A framed picture of shins with red nodules labeled 'ERYTHEMA NODOSUM', big red X, tagged 'IBD/SARCOID', upper-left.

WHAT IT ENCODES

Erythema nodosum (tender red shin nodules) is NOT a stigma of cirrhosis — it points to IBD, sarcoidosis, infection (strep, TB), or drugs. Don't pick it as a liver sign. The true cirrhosis stigmata come from hyperestrogenism and portal hypertension.

BOARD TRAP

"Erythema nodosum is a stigma of chronic liver disease" → wrong; it signals IBD/sarcoidosis, not cirrhosis.

2. Jaundice

WHAT YOU SEE

Yellow eyes/skin on the central figure labeled 'JAUNDICE'.

WHAT IT ENCODES

Jaundice (scleral icterus when bilirubin >2-3 mg/dL) reflects impaired bilirubin conjugation/excretion in advanced liver disease. A component of Child-Pugh scoring. Worsening jaundice signals decompensation.

3. Spider angiomata

WHAT YOU SEE

Central red star-shaped lesions on the chest labeled 'SPIDER ANGIOMATA'.

WHAT IT ENCODES

Spider angiomata (central arteriole with radiating vessels, blanch with pressure) result from hyperestrogenemia; numerous/large lesions correlate with severity and variceal risk. Found in the SVC distribution (face, upper chest, arms).

4. Palmar erythema

WHAT YOU SEE

Reddened palms labeled 'PALMAR ERYTHEMA' on the figure's hands.

WHAT IT ENCODES

Palmar erythema (red thenar/hypothenar eminences) is another hyperestrogenism sign. Non-specific (also pregnancy, hyperthyroidism, RA) but supports chronic liver disease in context.

5. Gynecomastia

WHAT YOU SEE

Enlarged breast tissue on the male figure labeled 'GYNECOMASTIA'.

WHAT IT ENCODES

Gynecomastia, testicular atrophy, and loss of body hair stem from altered estrogen/androgen balance (reduced hepatic clearance of estrogens). Spironolactone used for ascites can worsen gynecomastia.

6. Caput medusae

WHAT YOU SEE

Dilated periumbilical veins on the abdomen labeled 'CAPUT MEDUSAE'.

WHAT IT ENCODES

Caput medusae = dilated periumbilical veins from portal hypertension recanalizing the umbilical vein. A portosystemic collateral, like esophageal varices and rectal varices. Reflects elevated portal pressure.

7. Dupuytren contracture

WHAT YOU SEE

A clawed/contracted hand region labeled 'DUPUYTREN'.

WHAT IT ENCODES

Dupuytren contracture (palmar fascia thickening flexing the ring/little fingers) is associated with alcoholic liver disease (also diabetes, manual labor). Part of the stigmata cluster on exam.

8. HRS — stop diuretics + albumin

WHAT YOU SEE

A doctor holding a sign 'STOP DIURETIC, ALBUMIN 48H', center-right.

WHAT IT ENCODES

Hepatorenal syndrome is a DIAGNOSIS OF EXCLUSION: AKI with intense renal vasoconstriction, bland urine (no protein/casts), no improvement after stopping diuretics and a 48h albumin challenge (1 g/kg/day). Rule out hypovolemia, ATN, obstruction first. The reflex must be STOP diuretics + give albumin, never more diuretics.

BOARD TRAP

"Treat hepatorenal syndrome with more diuretics/furosemide" → wrong; STOP diuretics, give albumin challenge, then vasoconstrictors. Diuretics worsen the prerenal physiology.

9. Terlipressin + albumin

WHAT YOU SEE

A syringe labeled 'TERLIPRESSIN' constricting a kidney vessel, top-center-right.

WHAT IT ENCODES

After albumin, treat HRS with a vasoconstrictor + albumin: terlipressin (preferred, watch for ischemia/respiratory failure) or noradrenaline (ICU) or midodrine + octreotide where terlipressin unavailable. These reverse splanchnic vasodilation and improve renal perfusion as a bridge.

10. Transplant best (gold star)

WHAT YOU SEE

A hospital building 'TRANSPLANT BEST' with gold star, top-right.

WHAT IT ENCODES

Liver transplantation is the DEFINITIVE and best treatment for hepatorenal syndrome (gold star) — pharmacotherapy is only a bridge. HRS portends very poor survival without transplant. Renal function often recovers post-transplant.

11. Hepatopulmonary — platypnea

WHAT YOU SEE

A blue dyspneic figure 'PLATYPNEA ORTHODEOXIA' beside a 'BUBBLE-ECHO HEART', lower-right.

WHAT IT ENCODES

Hepatopulmonary syndrome = triad of liver disease + hypoxemia + intrapulmonary vascular dilations. Hallmark is PLATYPNEA-ORTHODEOXIA: dyspnea/desaturation WORSE when UPRIGHT (dilations are basal, more shunting standing). Diagnose with contrast (bubble) echocardiography showing delayed left-heart opacification. Definitive treatment is transplant.